

Riverside Medical Center

Department of Radiology & Medical Imaging

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RADIOLOGY REPORT

Patient Name:	SAMPLE PATIENT (De-identified)	MRN:	XXXX-0001
Date of Birth:	01/15/1958 Age: 68	Visit Date:	07/10/2026
Gender:	Male	Ordering MD:	Dr. A. Mitchell, MD

STUDY 1: CHEST X-RAY (PA AND LATERAL VIEWS)

Study Date:	07/09/2026	Accession #:	RXR-2026-04812
Modality:	X-Ray — 2 Views	Ordering MD:	Dr. A. Mitchell, MD
Radiologist:	Dr. P. Sharma, MD	Report Date:	07/09/2026 16:45 PM
Priority:	Routine	Clinical Site:	Outpatient Clinic

Clinical Indication:

New-onset shortness of breath on exertion in a 68-year-old male with T2DM and HTN. Evaluate for cardiac enlargement, pulmonary pathology, or pleural effusion.

Technique:

Posteroanterior (PA) and lateral chest radiographs obtained in full inspiration. Adequate technical quality. No prior imaging available for comparison.

Findings:

Lungs: Mild hyperinflation bilaterally with flattening of the diaphragm. No focal consolidation, mass, or cavity. Subtle increased interstitial markings at the bilateral lung bases may represent early interstitial changes or early pulmonary vascular congestion.

Heart: Cardiothoracic ratio measures 0.52, at the upper limit of normal. No gross cardiomegaly.

Mediastinum: Mediastinum is of normal width. Trachea is midline. No hilar lymphadenopathy.

Pleura: No pleural effusion. No pneumothorax identified on either view.

Osseous Structures: Mild degenerative changes at the thoracic spine. No acute fracture, lytic, or blastic lesion.

Soft Tissues: No significant soft tissue abnormality identified.

Impression:

- Mild pulmonary hyperinflation — findings consistent with early COPD or air trapping. Correlate clinically; pulmonary function testing (spirometry) is recommended.**
- Increased interstitial markings at bilateral lung bases — differential includes early congestive heart failure versus early interstitial lung disease. Echocardiogram recommended for further evaluation.**
- Cardiothoracic ratio at upper limit of normal — echocardiogram recommended to exclude early cardiomegaly or left ventricular dysfunction.**
- No acute cardiopulmonary process, pneumonia, or pneumothorax identified.**

Electronically signed: Dr. P. Sharma, MD (Radiologist) | 07/09/2026 16:45 PM | NPI: 2345678901

STUDY 2: ABDOMINAL ULTRASOUND — COMPLETE

Study Date:	07/09/2026	Accession #:	RUS-2026-02291
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Modality:	Ultrasound (Complete)	Ordering MD:	Dr. A. Mitchell, MD
Radiologist:	Dr. S. Reynolds, MD	Report Date:	07/09/2026 17:30 PM
Priority:	Routine	Clinical Site:	Outpatient Clinic

Clinical Indication:

Diabetic patient with elevated microalbumin-to-creatinine ratio (42 mg/g). Evaluate kidneys for diabetic nephropathy. Assess liver and abdominal organs.

Technique:

Real-time grayscale and color Doppler ultrasound of the abdomen performed in supine and decubitus positions following a 4-hour fast. Organs evaluated: liver, gallbladder, common bile duct, pancreas, spleen, bilateral kidneys, urinary bladder, and aorta.

Findings:

Liver: Normal in size and echogenicity. No focal hepatic lesion, abscess, or mass. No intrahepatic biliary ductal dilatation. No ascites adjacent to the liver.

Gallbladder: Gallbladder is well-distended, wall thickness 2 mm (normal). A single echogenic focus measuring 7 mm with posterior acoustic shadowing is consistent with cholelithiasis. No pericholecystic fluid. Murphy's sign negative.

Common Bile Duct: Normal caliber, measuring 4 mm. No intraluminal calculi or stricture.

Pancreas: Head, body, and tail visualized. Normal echogenicity. No pancreatic duct dilatation. No pancreatic mass or pseudocyst.

Spleen: Normal size (9.8 cm craniocaudal). Homogeneous echotexture. No focal lesion or splenomegaly.

Right Kidney: Measures 10.3 x 5.1 x 4.8 cm. Mildly increased cortical echogenicity relative to liver, suggesting early parenchymal changes. No hydronephrosis. No renal calculi identified. No cortical thinning. No renal mass.

Left Kidney: Measures 10.1 x 4.9 x 4.6 cm. Similar mild cortical echogenicity increase noted. No hydronephrosis, calculi, or mass. No perinephric fluid collection.

Urinary Bladder: Adequately distended with normal wall thickness (< 3 mm). No intraluminal lesion or calculus.

Abdominal Aorta: Normal caliber throughout (max AP diameter 2.1 cm). No aneurysmal dilatation. No significant plaque.

Free Fluid: No free intraperitoneal fluid identified.

Impression:

- 1. Cholelithiasis — single 7 mm gallstone; no sonographic features of acute cholecystitis. Recommend clinical correlation. Surgical referral if patient becomes symptomatic.
- 2. Bilateral mildly increased renal cortical echogenicity — in the clinical context of T2DM and elevated microalbumin-to-creatinine ratio, findings are consistent with early diabetic nephropathy. Nephrology referral recommended.
- 3. Normal hepatic parenchyma — no fatty infiltration or focal lesion identified.
- 4. Normal abdominal aorta — no abdominal aortic aneurysm.
- 5. No other acute intra-abdominal pathology.

Electronically signed: Dr. S. Reynolds, MD (Radiologist) | 07/09/2026 17:30 PM | NPI: 3456789012

STUDY 3: 12-LEAD ELECTROCARDIOGRAM (ECG)

Study Date:	07/09/2026	Accession #:	ECG-2026-00743
Modality:	12-Lead ECG	Ordering MD:	Dr. A. Mitchell, MD
Interpreted by:	Dr. P. Sharma, MD	Report Date:	07/09/2026 16:50 PM

Clinical Indication:

Shortness of breath on exertion. Rule out ischemia or arrhythmia.

ECG Parameters:

Rate	Rhythm	PR Interval	QRS Duration	QTc	Axis
78 bpm	Normal sinus	164 ms	94 ms	428 ms	+52°

Findings:

Normal sinus rhythm at 78 bpm. PR and QRS intervals within normal limits. QTc mildly prolonged at 428 ms (borderline; upper limit of normal 440 ms for males). No ST-segment elevation or depression. No T-wave inversion. No evidence of left ventricular hypertrophy by voltage criteria. No pathological Q waves. No bundle branch block.

Impression:

- 1. Normal sinus rhythm. No acute ischemic changes.
- 2. Borderline QTc prolongation (428 ms) — monitor; review medications that may prolong QT interval.
- 3. No evidence of LVH, bundle branch block, or arrhythmia.

Electronically signed: Dr. P. Sharma, MD (Radiologist) | 07/09/2026 16:50 PM | NPI: 2345678901

Report Verified by:	Dr. P. Sharma, MD Radiology	Date/Time:	07/09/2026 18:00 PM
Department Director:	Dr. R. Kapoor, MD Chief of Radiology	Accreditation:	ACR Accredited

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